MSC MARRIAGE AND FAMILY THERAPY

LECTURE LIBRARY · VOLUME 22 OF 28

MFT 770

Master Thesis

14 teaching weeks · 2 practicals

SEMESTER THREE

1.71 units · Prerequisite: MFT 705, 721

> “The thesis is not a bigger essay.”

One of 28 self-contained course books in the MSc Marriage and Family Therapy lecture library: a full taught semester per volume, with cases, practicals, assessment briefs, glossaries and model answers.

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MFT 770 — Master Thesis

MSc Marriage and Family Therapy · Semester Three (continuing) · 1.71 Units · 14 structured weeks + submission and defence · Prerequisite: MFT 705, 721

This volume is one of 28 course books that together cover the whole MSc Marriage and Family Therapy taught programme. Each book is written to carry a 14–16 week semester on its own: every week pairs a core idea with the practice that follows from it, a worked case, and a reflective task, and each book closes with an integration chapter, a glossary and a self-assessment paper with model answers.

The books are designed to be read straight through before the semester begins, revisited week by week, and used again in revision. Cross-references point to the other volumes where a topic is treated in more depth.

How to cite this volume

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A note on the case material

All families, couples and individuals in this book are fictional composites. They are written to teach pattern recognition, not to describe any real person. Nothing here replaces supervised clinical practice, professional registration, legal advice or medical care; legal content is orientation only and must be checked against the law of the relevant jurisdiction.

The lecture library

Twenty-eight self-contained course books carry the taught programme: eight in Semester One, seven in Semester Two, seven in Semester Three including the thesis, and six electives. The volume you are holding is marked in the margin.

Code	Course	Semester	Units
MFT 701	Fundamentals of Marriage and Family Therapy	One	1.71
MFT 702	Theories of Family Therapy	One	1.71
MFT 703	Human Development Across the Lifespan	One	1.71
MFT 704	Ethical and Legal Issues in Therapy	One	1.71
MFT 705	Research Methods in Marriage and Family Therapy	One	1.71
MFT 706	Clinical Practice and Techniques	One	1.71
MFT 707	Assessment and Diagnosis in Family Therapy	One	1.71
MFT 708	Cultural Competence in Therapy	One	1.71
MFT 709	Group Therapy Techniques	Two	1.71
MFT 710	Couples Therapy	Two	1.71
MFT 711	Child and Adolescent Therapy	Two	1.71
MFT 712	Crisis Intervention Strategies	Two	1.71
MFT 713	Substance Abuse and Family Dynamics	Two	1.71
MFT 714	Trauma-Informed Care	Two	1.71
MFT 715	Family Systems Theory	Two	1.71
MFT 716	Communication Skills in Therapy	Three	1.71
MFT 717	Psychopathology in Families	Three	1.71
MFT 718	Intimacy and Relationship Dynamics	Three	1.71
MFT 719	Family Life Cycle and Transitions	Three	1.71
MFT 720	Supervision and Consultation in Therapy	Three	1.71
MFT 721	Research Methodology	Three	1.71
MFT 722	Professional Development and Self-Care	Electives	1.71
MFT 723	Special Populations in Therapy	Electives	1.71
MFT 724	Family Law and Advocacy	Electives	1.71
MFT 725	Integrative Approaches in Therapy	Electives	1.71
MFT 726	Neurobiology of Relationships	Electives	1.71
MFT 727	Play Therapy Techniques	Electives	1.71
MFT 770	**Master Thesis**	Three	1.71

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Course leader's summary. The thesis is not a bigger essay. It is a supervised, ethical, original inquiry that a professional body would accept as evidence that you can generate knowledge, not only consume it. This book is the project management manual plus the academic guide.

How to use this book

Work the milestones in order; they are dated to protect you from the two classic disasters — a brilliant plan that cannot be completed, and a completed study that answers nothing. Bring the weekly artefact to supervision even if it is imperfect. Progress beats polish.

1. What the thesis is

Element	Requirement
Length	15,000–20,000 words (excluding appendices), or the institutional equivalent
Contribution	Original, however modest: new data, new analysis, a local validation, a qualitative account, a service evaluation
Methods	Quantitative, qualitative or mixed; must be defensible and feasible
Ethics	Institutional approval before any data collection
Supervision	Minimum fortnightly contact with a named supervisor
Output	Bound thesis, oral defence, and (encouraged) a publishable manuscript

What it is not. A literature review alone (that is a different degree component). A clinical case series without a systematic method. A replication of a manual with no question. A project that requires resources you do not have.

2. The fourteen structured weeks

Week	Milestone	Artefact due in supervision
1	Topic refinement and scope	One-page problem statement
2	Literature search strategy	Search log and 20 key references
3	Critical literature review draft	2,000-word draft
4	Research question and aims	PICO/SPIDER and aims table
5	Design selection	Design decision log
6	Measurement and materials	Instrument pack
7	Ethics application	Completed ethics form
8	Sampling and recruitment plan	Recruitment protocol and letters
9	Analysis plan	Pre-specified analysis plan

Week	Milestone	Artefact due in supervision
10	Pilot or feasibility work	Pilot findings and amendments
11	Data collection checkpoint	Recruitment and data quality report
12	Analysis progress	First results tables/coding frame
13	Findings and discussion drafting	Draft chapters 4–5
14	Full draft and defence preparation	Complete draft, viva questions

Timeline discipline. A part-time student should assume 12–18 months from registration to defence. Full-time students commonly need 9–12 months. If your plan assumes everything goes right, add 30%.

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3. Choosing a question you can finish

The three-circle test. A viable thesis sits where (1) you are curious, (2) the field needs it, and (3) you have access. If any circle is missing, choose a different question, not a bigger effort.

Good thesis questions in this field (examples).

1. What is the reliability and factor structure of the RDAS in a Nigerian clinical sample?
2. How do fathers describe their experience of being excluded from family therapy?
3. Does a four-session psychoeducation group reduce caregiver burden among families of people with psychosis?
4. What do couples report about the process of rebuilding trust after infidelity?
5. What predicts drop-out after a first family therapy appointment in a community clinic?
6. How do therapists in training describe the influence of their own family of origin on their clinical decisions?
7. Does a culturally adapted version of the speaker–listener technique improve communication in a small clinical series?

Questions to avoid. "The effect of family therapy on family functioning" (too broad); "Why is family therapy better than individual therapy?" (advocacy, not inquiry); anything requiring a sample you cannot access or an instrument you cannot obtain.

> **ARTEFACT**
> A one-page problem statement: what is unknown, why it matters, who it affects, and what you will produce.

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4. The literature review

Purpose. Not to summarise everything, but to establish (a) what is known, (b) what is contested, (c) what is missing, and (d) why your question follows.

Search discipline.

- At least three databases (PubMed, PsycINFO, Scopus, AJOL, Google Scholar as a supplement).
- Document the strategy: terms, limits, date, number of results, number retained.
- Use a reference manager from day one.
- Read systematic reviews first; they map the field faster than primary papers.

Structure of the chapter. Context → key constructs and definitions → what the evidence shows → what is contested → what is missing (the gap) → your question and aims.

The gap must be specific. "Few studies have examined X in Y population using Z method" — not "more research is needed."

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5. Design and methods

The rule. The method chapter must be detailed enough that a competent reader could replicate the study, and honest enough that they could see its limits.

Chapter contents.

1. Design and rationale.
2. Setting and context.
3. Participants: inclusion, exclusion, recruitment, consent, sample size justification.
4. Measures: source, psychometrics, scoring, adaptation and translation procedures.
5. Procedure: what happened, in what order, for how long.
6. Analysis: software, models, assumptions, missing data, coding approach, rigour procedures.
7. Ethical considerations: approval reference, consent, confidentiality, risk, data storage.
8. Researcher position and reflexivity.

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6. Ethics

Required before any data collection. A retrospective application is a serious breach.

Standard components. Purpose and design; participants and recruitment; consent process and documents; risks and benefits; confidentiality and data management; payment; dissemination; and procedures for distress or incidental findings.

Family-specific issues. Consent from guardians plus assent from minors; separate consent for each family member; managing the possibility that one member withdraws; protecting group confidentiality; and a protocol for disclosures of risk during data collection.

ARTEFACT

A complete ethics application with consent forms for an adult and a minor, and a risk protocol for a participant who discloses harm during an interview.

7. Data collection

Pilot first. Five to ten participants will reveal problems that a hundred will not forgive: unclear questions, unworkable recruitment, unusable audio, unrealistic time estimates.

Data quality. Check recordings the same week; transcribe promptly; keep a fieldwork log; record refusals and reasons (they are data).

Recruitment tactics that work in clinical settings. A named contact who can introduce you; a one-page information sheet; flexible times; a clear statement of what participation involves; and a follow-up call at 48 hours. Do not rely on posters.

8. Analysis

Quantitative. Follow the pre-specified plan; document deviations; check assumptions; report descriptives before inference; report effect sizes with confidence intervals; run sensitivity analyses; keep the analysis log current.

Qualitative. Transcribe verbatim; code systematically; keep memos; audit your coding; discuss divergence with a second coder; move from codes to themes with an explicit, documented logic; and present themes with data extracts (anonymised).

Mixed methods. Decide in advance how the strands will be integrated, and produce at least one joint display.

9. Writing the thesis

Standard structure.

Chapter	Content	Typical length
1	Introduction and background	2,000
2	Literature review	5,000
3	Methodology	3,500
4	Findings	4,000
5	Discussion	3,500
6	Conclusion and recommendations	1,000
—	References, appendices	—

Writing rules.

- Write the method first; it is the easiest chapter and it clarifies everything.
- Draft findings as soon as you have data; do not wait for "enough".
- Keep a "parking lot" file for ideas that belong elsewhere.
- Write 500 words a day rather than 5,000 in a panic.
- Read your work aloud; if you run out of breath, the sentence is too long.

Voice. Academic but human. First person is acceptable in reflective and qualitative sections; follow your institution's guidance. Avoid both inflated jargon and chatty informality.

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10. The supervision relationship

What your supervisor owes you. Regular contact; timely feedback; methodological and ethical guidance; honesty about the standard of your work; and reasonable availability.

What you owe your supervisor. Prepared meetings with an agenda and an artefact; drafts submitted on time; decisions recorded; and honesty about difficulties — especially delays.

Managing difficulty. If supervision is not working, raise it early with the programme director. A change of supervisor is legitimate and common; drifting for six months is not.

The meeting agenda template.

1. Progress since last meeting (three bullets).
2. Decisions needed today (the most important item first).
3. Problems I am stuck on.
4. What I will do before the next meeting.
5. Date of next meeting.

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11. The viva

What examiners test. Whether you understand your own study: why you chose it, what you did, what it means, and what it cannot support.

Preparation.

1. Re-read your thesis and mark every claim you could not defend.
2. Prepare a two-minute summary.
3. Prepare answers to the four standard questions: design, validity, analysis, conclusion.
4. Rehearse with a peer who is instructed to be difficult.
5. Bring a copy of your thesis, your analysis log and a pen.

In the room. Answer the question asked. Say "I don't know" when you don't. Distinguish what the data show from what you hope they show. Correct errors gracefully.

Common reasons for referral or failure. Design cannot answer the question; analysis misapplied; ethics breach; unacknowledged limitations; and evidence of work not being the candidate's own.

12. After the defence

Corrections. Make them promptly, with a tracking table mapping each examiner comment to the change made.

Dissemination. Submit a manuscript to a journal; present at a conference; give a plain-language summary to the participating service. Dissemination is part of the ethical contract with your participants.

Your data. Retain as required by your institution's policy, then destroy securely.

13. Project management

Risk	Early warning	Mitigation
Recruitment failure	Fewer than 50% of target by mid-point	Widen eligibility; add sites; adjust design (with supervisor and ethics approval)
Analysis paralysis	Repeated re-coding without writing	Fixed decision date; write the methods and limitations
Supervisor unavailability	Missed meetings	Written agenda; escalate early
Personal crisis	Missed deadlines	Tell the programme early; request extension with evidence
Scope creep	New questions each week	Freeze the question after Week 4; park ideas

The one-page dashboard. Question, milestones, current position, next action, date, supervisor appointment. Update weekly; it will save your degree.

14. Assessment

Component	Weight
Proposal (approved by panel)	10%
Ethics approval	Pass/fail gate
Thesis	70%
Oral defence	20%

Marking criteria (typical). Originality and significance; command of the literature; appropriateness and rigour of method; quality of analysis; critical discussion; ethical practice; presentation and referencing.

Appendix: thesis checklist

- ■ Question is specific, answerable, and stated in one sentence
- ■ Literature search documented and reproducible
- ■ Gap statement is specific, not "more research is needed"
- ■ Design justified against at least one alternative
- ■ Sample size justification provided
- ■ Ethics reference number included
- ■ Measures described with psychometric properties and any adaptation
- ■ Analysis plan pre-specified; deviations recorded and explained
- ■ Assumptions checked and reported
- ■ Effect sizes with confidence intervals
- ■ Findings presented before interpretation
- ■ Limitations specific and honest
- ■ Implications for practice are actionable
- ■ Referencing consistent and complete
- ■ Plain-language summary prepared for participants

Further reading

- Wisker, G. (2012). The Good Supervisor. Palgrave.
- Booth, W., Colomb, G., & Williams, J. (2016). The Craft of Research (4th ed.). Chicago.
- Swetnam, D., & Swetnam, R. (2009). Writing Your Dissertation. How To Books.
- Braun, V., & Clarke, V. (2022). Thematic Analysis: A Practical Guide. Sage.
- Field, A. (2018). Discovering Statistics Using IBM SPSS Statistics (5th ed.). Sage.

End of Semester Three. Continue to the electives: MFT 722 — Professional Development and Self-Care